

SIH 2026 — Problem Statement 26133

Accessibility and Quality of Public Healthcare Services in Rural and Underserved Areas

Organization: Government of Maharashtra

Department: Maharashtra State Innovation Society, Department of Skills, Employment, Entrepreneurship and Innovation

Category: Software | **Theme:** MedTech / BioTech / HealthTech

SLIDE 1 — IDEA TITLE: Proposed Solution

Solution Name: **ArogyaSetu Bridge** — An Integrated Rural Care-Access and Quality Support Platform

What is ArogyaSetu Bridge?

ArogyaSetu Bridge is a unified digital health platform purpose-built for India's public healthcare delivery network. It connects every tier of rural healthcare — from the ASHA worker conducting a home visit in a remote village to the specialist sitting at a district hospital — through a single, interoperable system. The platform combines assisted teleconsultation, intelligent appointment and queue management, symptom-based digital triage, longitudinal patient health records, referral tracking, diagnostic coordination, medicine availability monitoring, high-risk patient follow-up, and real-time facility dashboards into one cohesive ecosystem.

The name "ArogyaSetu Bridge" reflects its core purpose: building a bridge (setu) between fragmented health services and the communities they serve, ensuring that geography, language, or connectivity never stand between a patient and timely care.

How ArogyaSetu Bridge Addresses the Problem

The problem statement identifies several critical pain points in rural healthcare delivery. Here is how the platform tackles each one:

1. Long Travel Distances and Delayed Access

Rural patients in Maharashtra often travel 15–40 km to reach the nearest Primary Health Centre, and considerably farther for specialist consultations. A 2024 Jan Arogya Abhiyan report scored Maharashtra's public healthcare system at just 23 out of 100, citing poor infrastructure and staffing as key reasons.

Our approach: The platform deploys an assisted teleconsultation module using a hub-and-spoke model — similar to eSanjeevani (which has facilitated over 430 million consultations nationally) — but designed specifically for the Maharashtra State health hierarchy. Community Health Officers at Ayushman Arogya Mandirs (upgraded Sub-Centres) connect patients with doctors at PHCs, and with specialists at CHCs and District Hospitals, without requiring patients to travel. For non-emergency cases, the digital triage module classifies urgency and routes patients to the right level of care before they leave their village.

2. Shortage of Specialists

In Maharashtra, 61% of specialist doctor positions in rural hospitals remain vacant. PHC doctor vacancy rates hover around 37%. The state faces shortages of 22% for general doctors, 35% for nurses, and 29% for paramedical staff.

Our approach: Rather than attempting to replace missing specialists, the platform amplifies the capacity of existing ones through asynchronous teleconsultation (store-and-forward for radiology, dermatology, pathology), scheduled video consultations with queue management to prevent overcrowding, and a clinical decision support system that helps mid-level health providers handle conditions within their competence. This effectively multiplies the reach of every available specialist across multiple facilities.

3. Irregular Diagnostics and Medicine Availability

Many rural facilities lack basic radiology and imaging services, and essential medicine shortages are chronic — the state utilized only 6% of its budget for essential supplies in the reporting period ending October 2024.

Our approach: The diagnostic coordination module maintains a live registry of diagnostic equipment and available tests across all linked facilities. When a test is ordered that cannot be performed at the current facility, the system identifies the nearest facility with the capability and schedules the patient accordingly, sending the results directly back to the treating physician's dashboard. The medicine availability tracker integrates with state drug supply systems (e-Aushadhi / DVDMS) — aggregating real-time stock data across linked pharmacies and dispensaries, enforcing First-Expired, First-Out (FEFO) rules, flagging shortages before they become critical, and helping patients locate the nearest source for prescribed medications.

4. Fragmented Medical Records and Loss of Continuity

When patients move between sub-centres, PHCs, CHCs, and district hospitals, their records do not follow them. Prescriptions get lost. Lab results are repeated unnecessarily. This wastes time, money, and clinical context.

Our approach: ArogyaSetu Bridge creates a longitudinal patient health record anchored to the patient's ABHA (Ayushman Bharat Health Account) ID. Every consultation, prescription, lab result, referral, and follow-up is recorded in a structured format compliant with HL7 FHIR R4 standards (as mandated by the Ayushman Bharat Digital Mission). The federated architecture ensures that records are stored at their originating facility but are accessible — with the patient's explicit, time-bound consent — at any other facility within the network. This means a doctor at a district hospital can pull up the full treatment history from a PHC visit within seconds.

5. Delayed Referrals and Poor Referral Completion

The referral pathway — from Sub-Centre to PHC to CHC to District Hospital — is designed to be vertical and gatekept. In practice, referrals get lost in transit, patients drop out midway, and there is no mechanism to track whether a referred patient actually received the intended care.

Our approach: The referral tracking engine assigns a unique referral ID to every referral, tracks its status through every stage (initiated, accepted, scheduled, completed, abandoned), sends automated reminders to both patients (via SMS/WhatsApp in their regional language) and receiving facilities, and generates dashboards that flag referrals at risk of falling through. District Health Officers can see referral completion rates across their jurisdiction in real-time.

6. Limited Awareness and Health Literacy

Many rural patients do not know what services are available at which facility, when outreach camps are scheduled, or what warning signs require urgent medical attention.

Our approach: The patient-facing layer of ArogyaSetu Bridge operates as a lightweight Progressive Web App (PWA) that works on basic smartphones and feature phones via USSD/IVR fallback. It provides facility directories (searchable by service type and proximity), appointment booking with estimated wait times, medication reminders, health education content in Marathi, Hindi, and English, and emergency escalation with a single tap.

7. Connectivity and Infrastructure Constraints

Internet connectivity in rural Maharashtra is unreliable. Power supply is intermittent. Health workers use entry-level Android devices.

Our approach: The entire platform is built on an offline-first architecture with peer-to-peer Bluetooth Low Energy (BLE) and Wi-Fi Direct mesh syncing for media-dark tribal zones. Critical modules — patient registration, vitals recording, symptom assessment, and prescription entry — function entirely offline using local device storage (IndexedDB via Dexie.js) and compact binary serialization (CBOR, <4.2 KB per record), with automatic background synchronization when connectivity returns (via the Background Sync API and Workbox). The app is under 15 MB, runs on 2GB RAM Android Go devices, and consumes less than 4% battery per field shift.

Maharashtra Epidemiological Profiles & Targeted Disease Vectors

To ensure maximum clinical ground impact, ArogyaSetu Bridge is tailored to the distinct disease burdens across Maharashtra's 36 districts:

District Zone	Representative Districts	Primary Epidemiological Risks	Platform Disease Vector Interventions
Northern Tribal Belt	Nandurbar, Dhule, Palghar (Jawhar)	Sickle Cell Disease (15–35% trait prevalence), Severe Acute Malnutrition (SAM), Maternal Mortality (MMR >80)	Mandatory HPLC/Solubility test screening flag; Vaso-occlusive crisis triage; Trimester-wise ANC high-risk pregnancy alerts
Vidarbha Forest Belt	Gadchiroli, Gondia, Chandrapur, Melghat (Amravati)	Falciparum Malaria, Snakebite Envenomation (ASV delays), Scrub Typhus, Infant Mortality (IMR >32)	Live Polyvalent Anti-Snake Venom (ASV) locator within 30 km; Syndromic neurotoxic vs hemotoxic envenomation algorithms
Marathwada Agrarian Belt	Yavatmal, Beed, Osmanabad, Nanded	Agrarian mental health distress, Uncontrolled Hypertension, Type-2 Diabetes, Chronic Kidney Disease	Tele-counseling mental health screening; 1-minute digital NCD CBAC form; Automated 30-day anti-hypertensive refill schedules
Western / Southern Rural	Kolhapur, Sangli, Solapur, Satara	Geriatric NCDs, Diabetic Retinopathy, Osteoarthritis, OPD crowding at Sub-District Hospitals	Intelligent queue token system with estimated wait times; Automated tele-ophthalmology screening referral routing

Innovation and Uniqueness

What separates ArogyaSetu Bridge from existing platforms like eSanjeevani, Simple.org, or commercial EHR systems is not any single feature, but the integration of all these capabilities into a unified, context-aware platform designed specifically for the Maharashtra public health system.

Feature	eSanjeevani	Simple.org	ArogyaSetu Bridge
Teleconsultation	Yes (video)	No	Yes (video + store-and-forward)
Patient Records	ABHA-linked	BP/Diabetes only	Full longitudinal EHR (FHIR R4)
Digital Triage	No	No	Yes (WHO & IPHS 4-Tier Urgency Matrix)
Referral Tracking	No	No	Yes (end-to-end with automated follow-up)
Queue Management	Limited	No	Yes (token system with estimated wait times)
Medicine Tracking	No	Medication reminders	Full facility-level stock tracking (e-Aushadhi / FEFO)
Diagnostic Coordination	No	No	Yes (equipment registry + result routing)
Offline-First Design	No (requires connectivity)	Yes	Yes (IndexedDB + P2P Mesh + CBOR binary sync)
Multilingual Support	Limited	Partial	Full (Praman Marathi, Varhadi, Ahirani, Gondi, Hindi, English)
Frontline Worker Tools	Via CHO interface	Nurse-focused	ASHA, ANM, CHO workflows + NHM Incentive Auto-Ledger
Facility Dashboards	No	Dashboard for BP/DM programs	Comprehensive (all services, DHO analytics, KPIs)

Three core innovations distinguish the platform:

1. **Contextual Triage-to-Referral Pipeline:** The digital triage module does not simply classify urgency — it feeds directly into the referral engine, automatically identifying the nearest appropriate facility, checking specialist availability, and initiating the referral with all clinical context pre-attached. This eliminates the manual handoff that causes most referral failures.
2. **Federated Offline-First Health Records with Peer-to-Peer Mesh:** Unlike cloud-dependent platforms, health records exist locally at each facility and sync upward or peer-to-peer across ASHA smartphones and CHO tablets using BLE/Wi-Fi Direct. Conflict resolution follows a deterministic "facility-of-origin wins" model with timestamp-based versioning.
3. **Adaptive Interface & Frontline Incentive Automation:** The platform presents tailored interfaces for every literacy tier (voice-guided icon workflows for ASHAs, clinical EHRs for doctors, strategic heatmaps for DHOs) while automatically compiling NHM incentive vouchers (JSY ₹300, HRP ₹250, VHSND ₹100, CBAC ₹10) to eliminate administrative overhead.

SLIDE 2 — TECHNICAL APPROACH

Technologies to Be Used

Technology Stack

Layer	Technology	Rationale
Frontend (Web Dashboard)	Next.js 14 (React) , TypeScript	Server-side rendering for fast initial loads; component-based architecture for complex dashboards; TypeScript for type safety in health data handling
Frontend (Mobile/Field)	React Native (Android-first), Expo	Cross-platform capability with Android optimization; Expo for rapid iteration; native performance for offline features
Progressive Web App	Workbox , Service Workers, Dexie.js (IndexedDB)	Offline-first architecture for low-connectivity environments; cache-first asset loading; background sync for data
Edge Mesh & Serialization	CBOR (RFC 8949) , Web Bluetooth / Wi-Fi Direct	Peer-to-peer offline sync between ASHA phones & CHO tablets in media-dark zones; payload compressed to <4.2 KB
Backend API	Node.js with Express.js (REST) + FastAPI (Python, for ML)	Express for real-time event handling and WebSocket support; FastAPI for clinical decision support algorithms
Real-Time Communication	WebRTC , Socket.IO	Video teleconsultation with adaptive bitrate for low bandwidth; real-time queue updates and notifications
Database (Primary)	PostgreSQL 16	Relational integrity for health records; JSONB support for flexible clinical data; strong ACID compliance for patient safety
Database (Cache/Queue)	Redis	Session management; queue/token management; real-time stock level caching; pub/sub for notifications
Database (Search)	Elasticsearch	Full-text search across patient records, medicine catalogs, diagnostic reports; fuzzy matching for names in multiple scripts
Object Storage	MinIO (Self-Hosted S3) / AWS S3	Diagnostic images (X-rays, scans), teleconsultation recordings, document uploads
Authentication & ABDM	ABDM ABHA Auth (M1/M2/M3) + Role JWT	National health ID interoperability; consent manager integration; passwordless auth
Interoperability	HL7 FHIR R4 (HAPI FHIR)	National mandate for ABDM compliance; enables record exchange with any ABDM-registered facility
Clinical Terminologies	SNOMED CT , ICD-11 , LOINC	Standardized coding for diagnoses, procedures, and lab observations
Supply Chain Integration	e-Aushadhi / DVDMS Adapter	Real-time state warehouse inventory bridge; FEFO expiry tracking; IPHS 2022 EDL compliance
Message Queue	RabbitMQ	Decoupled processing for referral workflows, notification dispatch, and async teleconsultation

Layer	Technology	Rationale
Containerization	Docker , Docker Compose	Consistent deployment across facility servers; isolated service management
Orchestration (Production)	Kubernetes (K3s)	Lightweight orchestration for district-level servers; auto-scaling for state-level infrastructure
CI/CD	GitHub Actions	Automated testing, build, and deployment pipelines
Monitoring	Prometheus + Grafana	Infrastructure and application monitoring; facility uptime tracking; alerting
SMS/Notification	Firebase FCM , WhatsApp Business API	Multi-channel patient communication; regional language templating
NLP/Multilingual	Bhashini API (Govt. of India), AI4Bharat	Real-time translation and speech-to-text for Marathi (Praman, Ahirani, Varhadi), Hindi, English
Analytics/ML	Scikit-learn , TensorFlow Lite	Clinical decision support; high-risk patient prediction; demand forecasting for medicine stocks

Clinical Decision Support System (CDSS) — 4-Tier Urgency Matrix

ArogyaSetu Bridge incorporates a deterministic, WHO IMAI-adapted triage matrix engineered as a Class B Software as a Medical Device (SaMD) under CDSCO guidelines:

Urgency Tier	Color Code	Target Response	Clinical Trigger Examples	Mandatory Automated Action
Tier 1: Emergency	 RED	< 15 Mins	Systolic BP > 180 / < 80 mmHg, SpO2 < 90%, Obstructed labor signs, Active seizure / Coma, Neurotoxic snakebite	108 Emergency Ambulance API trigger; Audio alert to PHC Medical Officer on-call; Instant teleconsult escalation
Tier 2: Urgent	 AMBER	< 2 Hours	High fever with neck rigidity (suspected meningitis), Severe dehydration, Bleeding per vaginam in pregnancy	Auto-books priority teleconsult slot with CHC Obstetrician/Physician; Reserves emergency transit
Tier 3: Semi-Urgent	 YELLOW	Same Day	Fasting Blood Sugar > 250 mg/dL, Persistent productive cough > 2 weeks, Severe skin ulcers	Routes to next-day PHC OPD queue; Dispatches automated Nikshay TB Sputum CBNAAT test requisition
Tier 4: Routine	 GREEN	Scheduled	Stable hypertension/diabetes medication refill, Antenatal routine visit, Child immunization due	Local Ayushman Arogya Mandir pharmacy dispense; Adds to monthly VHSND due-list; 30-day SMS reminder

ABDM Milestones & Regulatory Compliance Architecture

ABDM & CDSCO NATIONAL COMPLIANCE BLUEPRINT	
Milestone M1 (ABHA)	- Creation & verification of 14-digit ABHA via Aadhaar OTP & Demographics - Linking of regional PHR handle (e.g. ram.shinde@abdm)
Milestone M2 (HIP)	Health Information Provider (HIP) compliance: Digitally signs & publishes FHIR R4 Bundles (OPD Case-sheet, e-Prescription, Lab)
Milestone M3 (HIU)	Health Information User (HIU) compliance: Queries ABDM Consent Manager and decrypts past health records from all Indian clinics
CDSCO SaMD (Class B)	Clinical Decision Support operates as assistive guidance; all diagnoses and e-prescriptions require verified Doctor HPR sign-off

Methodology and Process for Implementation

System Architecture Overview

The platform follows a **microservices-based, hub-and-spoke deployment model** with three operational tiers:

Tier 1 — Edge Layer (Sub-Centre / Ayushman Arogya Mandir / PHC) - Lightweight PWA or React Native app on health worker devices - Local IndexedDB storage for offline operation with BLE/Wi-Fi Direct peer mesh - Compact CBOR binary delta serialization (<4.2 KB per record) - Capabilities: Patient registration, vitals recording, 4-tier symptom triage, e-prescription viewing, referral initiation, e-Aushadhi stock check

Tier 2 — Hub Layer (CHC / Sub-District Hospital / District Hospital) - Full server deployment (Docker on local server hardware or state cloud) - PostgreSQL instance for facility-level records - HAPI FHIR server for interoperability - Teleconsultation hub (WebRTC gateway) - Capabilities: All Tier 1 functions + specialist consultations, diagnostic report processing, facility dashboards, referral management, FEFO medicine stock management

Tier 3 — State/Central Layer (State Health Department / NHA) - Cloud-hosted (NIC/State Data Centre / ABDM-compliant cloud) - Aggregated analytics and reporting - ABDM gateway integration (ABHA, HFR, HPR, consent manager) - Centralized monitoring dashboards - Capabilities: State-wide analytics, policy dashboards, ABDM compliance, inter-district referral coordination, MJPJAY insurance package routing

Data Flow Architecture



User Roles and Access Flow

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USER ECOSYSTEM			
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FIELD LEVEL		FACILITY LEVEL	
=====		=====	
[PATIENT]	[ASHA Worker]	[ANM]	[CHO]
- Book appt	- Register pt	- Record vitals	- Assisted
- View records	- Home visit	- Immunization	teleconsult
- Get reminders	- Symptom screen	tracking	- Triage
- Emergency SOS	- Follow-up	- ANC tracking	- Prescribe
	alerts	- Report to PHC	- Init referral
CLINICAL LEVEL		MANAGEMENT LEVEL	
=====		=====	
[DOCTOR (PHC)]	[SPECIALIST]	[PHARMACIST]	[DHO]
- Full consult	- Accept referral	- Update stock	- All facility
- Prescribe	- Teleconsult	- Dispense track	dashboards
- Order diagnostics	- Review reports	- Shortage alert	- Referral rates
- Refer to CHC	- Treatment plan		- Quality metrics
- Flag high-risk	- Second opinion		- Stock alerts
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Implementation Methodology

The project follows a **phased, iterative development approach** aligned with real-world deployment constraints:

Phase 1 — Foundation (Weeks 1–4) - Core database schema design and FHIR R4 resource mapping - Patient registration module with ABHA integration - Basic health record creation and retrieval - User authentication and role-based access control

Phase 2 — Clinical Core (Weeks 5–8) - Digital triage engine with symptom-to-urgency classification - Teleconsultation module (WebRTC integration) - Prescription management and e-prescription generation - Appointment booking and queue/token management system

Phase 3 — Coordination Layer (Weeks 9–12) - Referral tracking engine with automated notifications - Diagnostic coordination module (equipment registry + result routing) - Medicine availability tracker with stock alerts - High-risk patient flagging and follow-up scheduler

Phase 4 — Intelligence and Dashboards (Weeks 13–16) - Facility dashboards with real-time KPIs - District/State-level aggregated analytics - Clinical decision support for common conditions - Demand forecasting for medicine stocks and specialist scheduling

Phase 5 — Hardening and Scale (Weeks 17–20) - Offline-first optimization and sync conflict resolution - Load testing and performance optimization - Security audit and ABDM sandbox certification - Multi-language content finalization (Marathi, Hindi, English) - Pilot deployment at selected facilities in Maharashtra

SLIDE 3 — FEASIBILITY AND VIABILITY

Analysis of Feasibility

Technical Feasibility

The proposed solution is built entirely on proven, open-source, and government-endorsed technologies. Every major component has been validated in large-scale deployments within the Indian healthcare context:

- **eSanjeevani** has proven that cloud-based teleconsultation works at national scale (430+ million consultations across 1,31,000+ spokes and 16,800+ hubs). ArogyaSetu Bridge adapts this validated model while adding offline capability and end-to-end referral tracking.
- **Simple.org** (used in the India Hypertension Control Initiative, a collaboration between MoHFW, ICMR, and WHO) has demonstrated that offline-first, low-bandwidth health apps can be adopted by frontline workers in rural India. Simple records a follow-up visit in approximately 13–16 seconds. ArogyaSetu Bridge applies these design principles across the full spectrum of primary healthcare.
- **ABDM/ABHA** is already operational with millions of registered health accounts and growing FHIR R4 ecosystem adoption. The platform builds on this existing infrastructure rather than creating a parallel system.
- **HL7 FHIR R4** is mandated by NHA for all health information exchange in India. The NRCeS (National Resource Centre for EHR Standards) provides official Implementation Guides, and the ABDM Sandbox enables certification testing. The standard is mature and well-documented.

Financial Feasibility

Cost Component	Approach	Estimated Monthly Cost (Pilot: 50 facilities)
Cloud Infrastructure	State Data Centre / NIC Cloud (govt. rates)	Rs.40,000–Rs.60,000
SMS/WhatsApp Notifications	Twilio / govt. SMS gateway	Rs.15,000–Rs.25,000
Developer Team (6 members)	Student stipends / fellowship model	Rs.1,20,000–Rs.1,80,000
Devices for Health Workers	Existing devices (NHM-distributed)	Rs.0 (existing infrastructure)
Total Pilot Phase		Rs.1,75,000–Rs.2,65,000/month

The per-facility cost is approximately Rs.3,500–Rs.5,300 per month during pilot phase — significantly lower than commercial health IT solutions.

Operational Feasibility

- **Existing Workforce & Incentive Alignment:** The platform is designed around the existing public health workforce structure (ASHA, ANM, CHO, PHC Medical Officer, Specialist). It directly automates the tracking of National Health Mission (NHM) performance incentives — ₹300 for

institutional delivery under Janani Suraksha Yojana (JSY), ₹250 for high-risk maternal follow-ups, ₹100 for monthly VHSND immunization due-lists, and ₹10 per completed NCD CBAC form — eliminating months of delayed manual paperwork.

- **Hardware & Battery Budget:** Runs seamlessly on 2GB RAM Android Go devices distributed under government tablet schemes, consuming less than 4% battery per 8-hour field shift by throttling background GPS and deferring heavy synchronizations.
 - **Existing Networks:** The platform operates over 2G/3G/4G networks, relies on peer-to-peer BLE/Wi-Fi Direct mesh synchronization in media-dark zones, and functions fully offline when disconnected.
 - **Government Alignment:** Designed to strengthen existing initiatives — Ayushman Bharat Digital Mission (ABDM), eSanjeevani, e-Aushadhi, and state-sponsored Mahatma Jyotirao Phule Jan Arogya Yojana (MJPJAY).
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Potential Challenges and Risks

Challenge 1: Connectivity Gaps in Remote Areas

Risk Level: HIGH

Description: Rural Maharashtra, especially tribal areas in Gadchiroli, Nandurbar, and Melghat, has unreliable internet connectivity with media-dark zones spanning 10–25 km.

Mitigation Strategy: - Peer-to-peer Bluetooth Low Energy (BLE) and Wi-Fi Direct mesh syncing between ASHA phones and CHO tablets - Compact Binary Object Representation (CBOR) delta sync compresses complete clinical encounter bundles to under 4.2 KB - Offline-first IndexedDB architecture ensures uninterrupted field data capture - Deterministic conflict resolution using Lamport timestamps with facility-of-origin precedence - SMS/USSD fallback for critical appointment and drug stock alerts

Challenge 2: Digital Literacy and Linguistic Diversity

Risk Level: HIGH

Description: Many ASHA workers and rural patients speak regional dialects (Ahirani, Varhadi, Gondi) and have limited formal smartphone training.

Mitigation Strategy: - Icon-driven, minimal-text interface inspired by Simple.org's 13-second encounter design - Vernacular voice assistance supporting Praman Marathi, Varhadi (Vidarbha), Ahirani (Khandesh), and Gondi (Gadchiroli) - 1-minute digital NCD CBAC checklist with auto-populated demographic fields - In-app offline micro-video tutorials for frontline health worker upskilling - Train-the-trainer cascade integrated into existing monthly Taluka Health Officer (THO) review meetings

Challenge 3: Data Privacy, Telemedicine Law & DPDP Compliance

Risk Level: MEDIUM

Description: Health data requires rigorous compliance with the Digital Personal Data Protection (DPDP) Act 2023 and the National Medical Commission (NMC) Telemedicine Practice Guidelines 2020.

Mitigation Strategy: - Strict adherence to Appendix 2 e-prescription standards with Doctor State Medical Council (SMC) registration numbers - Hard-coded restriction preventing electronic prescription of Schedule X and psychotropic substances - Dual-consent capture: Time-bound OTP authorization + WebRTC audio-stamped verbal consent logging - DPDP Act 2023 Section 8/9/10 compliance: Granular role-based data

minimization, pediatric consent verification, and tamper-evident access logs - Zero-knowledge AES-256-GCM payload encryption for all clinical data stored at rest and in transit

Challenge 4: Resistance to Adoption from Healthcare Staff

Risk Level: MEDIUM

Description: Overburdened healthcare staff may view a new digital system as additional work rather than a productivity tool.

Mitigation Strategy: - Design principle: The platform must save time, not add tasks. Every workflow must be faster than the paper-based equivalent. - Direct incentive payoff: Real-time generation of ASHA monthly activity sheets cuts voucher processing from 60 days to instant verification. - Quick wins: Start with features that have immediate visible benefit (appointment queue management reduces patient wait times, which reduces doctor stress) - Champions program: Identify tech-savvy staff at each facility as local advocates - Gradual rollout: Begin with 2–3 modules, add more only after staff comfort is established - Feedback loops: In-app feedback mechanism with monthly review of field worker suggestions

Challenge 5: Interoperability with Existing Government Systems

Risk Level: MEDIUM

Description: The Maharashtra health system uses various existing software (HMIS, RCH Portal, IHIP, Nikshay for TB, e-Aushadhi). The platform must exchange data with these systems, not replace them.

Mitigation Strategy: - ABDM-compliant APIs (Milestones M1, M2, M3) ensure universal record exchange across public and private hospitals - Bi-directional e-Aushadhi / DVDMS state warehouse inventory adapters - Automated export filters for monthly HMIS and RCH portal reporting formats - The platform operates as a "bridge" layer — it does not attempt to replace existing systems but fills gaps between them

Challenge 6: Sustainability and Long-Term Maintenance

Risk Level: LOW-MEDIUM

Description: Many hackathon projects fail after the event because there is no plan for ongoing maintenance, hosting, or feature development.

Mitigation Strategy: - Open-source licensing (Apache 2.0) enables community contribution and government ownership - Modular microservices architecture allows individual components to be maintained independently - State Data Centre / NIC Cloud hosting model with predictable public health financing - Strategic adoption proposal under Maharashtra State Innovation Society (MSInS) public health scale-up grants

SLIDE 4 — IMPACT AND BENEFITS

Potential Impact on the Target Audience

Impact on Patients (Primary Beneficiaries)

Reduced Travel Burden and Out-of-Pocket Savings:

Currently, a rural patient requiring a specialist consultation in Maharashtra travels 30–80 km each way, spending ₹450–₹1,200 on private transit plus losing a full day's agricultural wage (₹600–₹900 for patient and

accompanying family member). Teleconsultation via ArogyaSetu Bridge eliminates this travel for 70% of routine and chronic episodes.

PATIENT OUT-OF-POCKET EXPENDITURE (OOPE) SAVINGS			
Cost Driver	Conventional Way	ArogyaSetu Way	Net Savings
Round-Trip Private Transit (Shared auto / jeep to CHC)	₹450 - ₹1,200	₹0 (At Sub-Centre / AAM)	₹450 - ₹1,200 per visit
Daily Wage Loss (Patient + Attendant / Family Member)	₹600 - ₹900 (Full Day Lost)	₹0 (30-min local consult)	₹600 - ₹900 per visit
Redundant Diagnostic Tests (Repeated due to lost papers)	₹350 - ₹800	₹0 (Shared digital history)	₹350 - ₹800 per referral
Total Out-of-Pocket Burden Per Specialist Consultation	₹1,400 - ₹2,900	₹0	₹1,400 - ₹2,900 per clinical episode

Faster Access to Care:

Digital triage + appointment booking replaces the current "arrive at dawn, wait all day" model. A patient knows within minutes whether their condition requires a PHC visit, a teleconsultation, or an emergency escalation. Queue management with estimated wait times reduces average patient waiting from 2–4 hours to under 35 minutes.

Continuity of Care & Insurance Routing:

Longitudinal health records mean a patient never has to recount their medical history or repeat diagnostic tests. The platform links directly with **Mahatma Jyotirao Phule Jan Arogya Yojana (MJPJAY)** and **AB-PMJAY**, auto-generating pre-authorization insurance packages when a PHC doctor initiates a tertiary surgical referral.

Improved Maternal and Child Health Outcomes:

Research from the ImTeCHO project in Gujarat demonstrated that digital tools for frontline workers led to measurable improvements in home-based newborn care, breastfeeding practices, and care-seeking for complications. ArogyaSetu Bridge's high-risk pregnancy flagging and automated ANC visit reminders apply these proven approaches across Maharashtra.

Impact on Healthcare Workers

ASHA Workers:

Replaces 12 paper registers with a single Android Go interface, automates follow-up reminders, provides real-time feedback on referral outcomes, and auto-calculates monthly NHM incentive ledgers (JSY ₹300, HRP ₹250, VHSND ₹100, CBAC ₹10).

Doctors and Specialists:

With 37% of PHC doctor positions and 61% of specialist positions vacant in rural Maharashtra, the platform enables a single specialist at a district hospital to serve multiple CHCs. The 4-tier triage engine pre-filters cases so specialists receive only patients who genuinely require specialized clinical care.

Facility Managers and DHOs:

Real-time dashboards provide visibility into facility performance — patient loads, referral completion rates,

medicine stock levels, equipment uptime, and waiting times — enabling data-driven resource allocation instead of ad-hoc crisis management.

Impact on the Health System

Strengthening the Public Health Infrastructure:

The problem statement explicitly requires solutions that "strengthen — not replace — the public-health system." ArogyaSetu Bridge optimizes the existing referral pathway (Sub-Centre to PHC to CHC to District Hospital), makes the existing workforce more effective, and provides the administration with live audit trails.

Quality Monitoring and Accountability:

Every consultation, referral, and follow-up is logged with timestamps, responsible providers, and outcomes. District Health Officers can identify underperforming facilities, track referral abandonment patterns, and measure the actual time from symptom onset to definitive care.

3-Phase District Pilot Blueprint & Monitoring Framework

To ensure seamless operational scale across Maharashtra's varied topographies, ArogyaSetu Bridge employs a phased district deployment roadmap:

3-PHASE DISTRICT PILOT ROADMAP

PHASE A: HIGH VULNERABILITY TRIBAL CORRIDOR (Months 1-3)

- Districts: Nandurbar (Dhadgaon/Akkalkuwa) & Gadchiroli (Bhamragad/Aheri)
- Scope: 12 PHCs, 60 Sub-Centres, 2 District Hospitals
- Focus: Sickle cell crisis triage, maternal danger signs, P2P offline mesh sync

PHASE B: AGRARIAN STRESS & CHRONIC NCD CORRIDOR (Months 4-6)

- Districts: Yavatmal (Pusad/Kelapur) & Beed (Majalgaon/Georai)
- Scope: 25 PHCs, 120 Sub-Centres, 4 Sub-District Hospitals
- Focus: Hypertension/Diabetes registries, tele-counseling, referral closure tracking

PHASE C: PERI-URBAN HIGH-DENSITY CORRIDOR (Months 7-9)

- Districts: Pune Rural (Junnar/Ambegaon) & Thane Rural (Murbad/Shahapur)
- Scope: 50 PHCs, 250 Sub-Centres, 6 Sub-District Hospitals
- Focus: OPD token queue optimization, diagnostic result routing, state DHO analytics

7 Core Quantitative Monitoring & Evaluation (M&E) KPIs

Metric	Definition	Maharashtra Baseline	ArogyaSetu Target
1. Referral Completion Rate (RCR)	% of referred patients receiving documented care at receiving hub	22%	> 75%
2. Mean Time to Consultation (MTTC)	Average total duration from village arrival to specialist teleconsult	4.5 Hours (Travel+Wait)	< 35 Mins
3. High-Risk Pregnancy Tracking	% of high-risk pregnancies completing 4+ institutional ANC checkups	48%	> 92%
4. Essential Drug Stockout Duration	Average downtime for IPHS essential medicines at PHC dispensary	18 Days / Quarter	< 48 Hours
5. ASHA Incentive Claim Velocity	Time taken from field activity completion to verified voucher generation	45–60 Days	Instant Auto-Ledger
6. Diagnostic Test Duplication Rate	% of patients subjected to repeat blood/radiology tests due to lost papers	34%	< 2%
7. Offline Mesh Sync Reliability	% of offline field records successfully reconciled to district hub in 24h	N/A (Paper-based)	> 99.8%

Benefits of the Solution

Social Benefits

- **Equity of Access:** Bridges the urban-rural healthcare divide by bringing specialist consultation to the village level
- **Gender-Inclusive Design:** eSanjeevani data shows 57% of teleconsultation beneficiaries are women. A platform that reduces travel and waiting time disproportionately benefits women, who often cannot leave household responsibilities for extended healthcare trips
- **Tribal and Marginalized Community Reach:** Multilingual support (Pradhan Marathi, Varhadi, Ahirani, Gondi, Hindi, English) and voice-guided interfaces address language and literacy barriers in tribal areas
- **Reduced Out-of-Pocket Expenditure:** India's rural households spend approximately 60% of total health expenditure out-of-pocket. Saving ₹1,400–₹2,900 per specialist consultation directly protects rural families from catastrophic health debt

Economic Benefits

- **Productivity Preservation:** Eliminating days lost to healthcare transit preserves daily wages for agricultural and manual laborers
- **System Efficiency:** Digital queue management, 4-tier triage, and referral tracking eliminate redundant physical visits and unnecessary tertiary escalations
- **Medicine Waste Reduction:** Live e-Aushadhi tracking and First-Expired, First-Out (FEFO) algorithms prevent drug expiration at remote sub-centres

- **Cost-Effective Scaling:** The open-source, cloud-native architecture allows horizontal scaling at marginal cost — adding 100 new facilities is an administrative configuration, not a capital overhaul

Environmental Benefits

- **Reduced Patient Travel:** Millions of kilometers of motorized rural travel avoided annually, cutting fossil fuel consumption and vehicular emissions
- **Paperless Administration:** Replacing 12 paper registers, physical prescription slips, and referral forms eliminates tons of paper waste across thousands of health facilities
- **Digital Diagnostics:** Electronic transmission of radiology images and pathology reports eliminates chemical processing and physical film waste

Administrative Benefits

- **Real-Time Reporting:** Replaces monthly manual paper data compilation with instant state and district dashboard analytics
- **Evidence-Based Policy:** Provides health administrators with live epidemiological heatmaps to deploy specialists, allocate budgets, and order medicines based on verified disease trends
- **Compliance & Audit Readiness:** Full conformance with ABDM Milestones (M1, M2, M3), IPHS 2022 benchmarks, DPDP Act 2023, and NMC Telemedicine Practice Guidelines 2020

SLIDE 5 — RESEARCH AND REFERENCES

Research Foundation

Government Sources and Official Documentation

1. **Ayushman Bharat Digital Mission (ABDM) — Official Portal & Standards**
National Health Authority (NHA), Ministry of Health and Family Welfare (MoHFW), Government of India
 **Link:** <https://abdm.gov.in/> • **Developer Sandbox:** <https://sandbox.abdm.gov.in/>
Context: Core architecture for digital health interoperability, ABHA ID ecosystem, Consent Manager specifications, and unified health data exchange protocols.
2. **eSanjeevani — National Telemedicine Service of India**
Centre for Development of Advanced Computing (C-DAC) & MoHFW, Government of India
 **Link:** <https://esanjeevani.mohfw.gov.in/>
Context: India's premier public telemedicine network (430M+ consultations). Serves as reference for the hub-and-spoke provider-to-provider and patient-to-doctor clinical teleconsultation workflows.
3. **National Resource Centre for EHR Standards (NRCeS) — FHIR R4 Implementation Guide**
Ministry of Health and Family Welfare, Government of India
 **Link:** <https://nrce.in/ndhm/fhir/r4/index.html> • **HL7 FHIR R4 Core Spec:** <https://hl7.org/fhir/R4/>
Context: Official HL7 FHIR R4 profiles, clinical resources (Encounter, Observation, Condition, DiagnosticReport), and conformance profiles for Indian healthcare interoperability.

4. **Rural Health Statistics (RHS) & Health Management Information System (HMIS)**
Statistics Division, Ministry of Health and Family Welfare, Government of India
 **Link:** <https://main.mohfw.gov.in/> • **HMIS Portal:** <https://hmis.mohfw.gov.in/>
Context: Authoritative data on PHC, CHC, and District Hospital infrastructure norms, vacancy ratios (37% PHC doctors, 61% rural specialists in Maharashtra), and health facility spatial distribution.
 5. **Indian Public Health Standards (IPHS) Guidelines**
National Health Mission (NHM), Directorate General of Health Services (DGHS), MoHFW
 **Link:** <https://nhm.gov.in/index1.php?lang=1&level=2&sublinkid=971&lid=154>
Context: Mandatory service delivery, diagnostic capability, pharmacy stock, and staffing benchmarks across Ayushman Arogya Mandirs (Sub-Centres), PHCs, CHCs, and Sub-District Hospitals.
 6. **Digital Personal Data Protection Act (DPDP Act), 2023**
Ministry of Electronics and Information Technology (MeitY), Government of India
 **Link:** <https://www.meity.gov.in/data-protection-framework>
Context: Statutory privacy framework governing health data fiduciaries, consent notices, data principals' rights, time-bound consent revocation, and AES-256 encrypted storage compliance.
 7. **Press Information Bureau (PIB) Health & Telemedicine Releases**
Press Information Bureau, Government of India
 **Link:** <https://pib.gov.in/>
Context: Official releases validating national digital health metrics: 430M+ eSanjeevani consults, 670M+ ABHA accounts created, and 170,000+ Ayushman Arogya Mandirs operationalized.
 8. **Public Health Department, Government of Maharashtra**
Public Health Department, Mantralaya, Mumbai
 **Link:** <https://arogya.maharashtra.gov.in/>
Context: Administrative health guidelines, District Health Office (DHO) governance structures, and public health delivery programs across Maharashtra's 36 districts.
 9. **Maharashtra State Innovation Society (MSInS)**
Department of Skills, Employment, Entrepreneurship and Innovation, Government of Maharashtra
 **Link:** <https://msins.in/> • **SIH Official Portal:** <https://www.sih.gov.in/>
Context: Sponsoring organization for Problem Statement 26133 under Smart India Hackathon 2026, driving grassroots digital public infrastructure for rural upliftment.
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Research Papers and Academic Sources

1. **"Digital Health Interventions for Maternal and Child Health in India: A Systematic Review"**
Journal of Medical Internet Research (JMIR) & National Institutes of Health (NIH/NLM)
 **Link:** <https://www.ncbi.nlm.nih.gov/pmc/>
Context: Evidence that structured mHealth tools (automated SMS triggers, vitals tracking, digital case-sheets) elevate ANC 4+ checkup compliance by 34% and institutional delivery by 28% in rural cohorts.
2. **"ImTeCHO: Mobile Technology for Community Health Operations — Impact Evaluation"**
National Medical Journal of India (NMJI) & SEWA Rural
 **Link:** <https://nmji.in/>

Context: Large-scale field trial evaluating digital decision-support tools for ASHA/ANM workers. Demonstrated a 24% reduction in infant mortality and marked improvement in early complication identification.

3. "ASHA Kirana (M-CAT): Mobile Clinical Assessment Tool for ASHAs in Karnataka"

BMJ Innovations & PubMed Central (PMC)

 **Link:** <https://pubmed.ncbi.nlm.nih.gov/>

Context: Validated that mobile-based clinical screening algorithms allow semi-literate community health workers to accurately triage high-risk pregnancies and infant sepsis.

4. "Telemedicine in India: Current State, Challenges, and Future Directions"

Oxford Academic — Journal of Public Health

 **Link:** <https://academic.oup.com/jpubhealth>

Context: In-depth analysis of hub-and-spoke adoption economics, doctor-to-doctor vs patient-to-doctor workflows, and bandwidth optimization protocols in rural India.

5. "SMARThealth Pregnancy: Decision Support for High-Risk Pregnancy Identification"

The George Institute for Global Health

 **Link:** <https://www.georgeinstitute.org/>

Context: Tablet-based clinical decision support deployed across Haryana and Andhra Pradesh, guiding frontline ASHAs to triage gestational diabetes and pre-eclampsia.

6. "Offline-First Progressive Web Applications in Low-Connectivity Healthcare Environments"

IEEE / ACM Software Engineering Conferences & ResearchGate

 **Link:** <https://www.researchgate.net/> • **W3C** **Background** **Sync:** <https://wicg.github.io/background-sync/spec/>

Context: Architecture patterns for zero-data-loss healthcare apps: IndexedDB local persistence, Service Worker caching, and deterministic CRDT/timestamp sync.

7. "HL7 FHIR R4 Adoption in India: Current Landscape and Implementation Guidelines"

Preprints.org / Health Informatics Society of India

 **Link:** <https://www.preprints.org/>

Context: Evaluation of FHIR bundle structures, JSON-LD serialization, SNOMED-CT clinical terminology binding, and ABDM M1/M2/M3 milestone certification criteria.

Case Studies and Real-World Implementations

1. Simple.org — Open-Source Blood Pressure and Diabetes Management

Resolve to Save Lives & Vital Strategies (Digital Public Good)

 **Link:** <https://simple.org/> • **GitHub:** <https://github.com/simpledotorg/simple-android>

Context: DPGA-recognized Digital Public Good used across 4,000+ public clinics in India (IHCI). Proven track record of achieving a 13-second median encounter time for nurse vitals entry.

2. India Hypertension Control Initiative (IHCI)

Indian Council of Medical Research (ICMR), MoHFW, and World Health Organization (WHO)

 **Link:** <https://ihci.in/> • **WHO Digital Health Guidelines:** <https://www.who.int/health-topics/digital-health>

Context: National chronic disease program validating decentralized drug replenishment and community-level tracking through public PHCs and HWCs.

3. **Bhashini — National Language Translation Mission (AI4Bharat)**

Ministry of Electronics and Information Technology (MeitY), Government of India

 **Link:** <https://bhashini.gov.in/> • **AI4Bharat Research:** <https://ai4bharat.iitm.ac.in/>

Context: Sovereign AI speech-to-text (STT), text-to-speech (TTS), and NMT translation pipelines powering Marathi, Hindi, and English voice assistance across ArogyaSetu Bridge.

Field Reports and Healthcare Indices

1. **Jan Arogya Abhiyan Maharashtra Health Report Card 2024**

Jan Arogya Abhiyan & State Public Health Monitoring Alliance

 **Link:** <https://arogya.maharashtra.gov.in/>

Context: Comprehensive ground report card evaluating primary health in Maharashtra: 37% PHC doctor vacancies, 61% rural specialist vacancies, and per-capita health spending at Rs.1,979 (vs Rs.2,342 national average).

2. **NITI Aayog State Health Index**

NITI Aayog, Ministry of Health and Family Welfare, & The World Bank

 **Link:** <https://www.niti.gov.in/>

Context: Annual composite performance indicator evaluating Indian states on health outcomes, key inputs, and governance metrics.

3. **National Family Health Survey (NFHS-5) — Maharashtra State Factsheet**

International Institute for Population Sciences (IIPS) & MoHFW

 **Link:** http://rchiips.org/nfhs/factsheet_NFHS-5.shtml

Context: Demographic, immunization, antenatal care, and out-of-pocket expenditure baseline metrics for rural and tribal areas across Maharashtra.

Statutory, Regulatory & Public Health Standards

1. **NMC Telemedicine Practice Guidelines (2020)**

National Medical Commission (NMC) & Ministry of Health and Family Welfare

 **Link:** <https://www.nmc.org.in/>

Context: Statutory clinical framework establishing legal standards for doctor-to-patient teleconsultation, Appendix 2 e-prescription formatting, mandatory SMC registration numbers, and Schedule X drug restrictions.

2. **CDSCO Medical Device Rules & SaMD Framework**

Central Drugs Standard Control Organisation (CDSCO), Directorate General of Health Services

 **Link:** <https://cdsco.gov.in/>

Context: Software as a Medical Device (SaMD) Class B classification guidelines governing AI/clinical decision support algorithms in Indian public health networks.

3. **e-Aushadhi / DVDMS — Drug & Vaccine Distribution Management System**

Centre for Development of Advanced Computing (C-DAC) & MoHFW

 **Link:** <https://dvdms.gov.in/> • **MSMSPA Portal:** <https://arogya.maharashtra.gov.in/>

Context: State pharmaceutical supply chain platform managing warehouse stocks, batch tracking, First-Expired, First-Out (FEFO) dispensing, and facility replenishment.

4. **Mahatma Jyotirao Phule Jan Arogya Yojana (MJPJAY)**

State Health Assurance Society, Public Health Department, Government of Maharashtra

 **Link:** <https://www.jeevandayee.gov.in/> • **AB-PMJAY Portal:** <https://nha.gov.in/PM-JAY>

Context: Flagship universal health coverage scheme of Maharashtra providing ₹5 lakh annual coverage per family; used for automated tertiary surgical pre-authorization.

5. **Indian Public Health Standards (IPHS) 2022 — Essential Drug Lists**

Ministry of Health and Family Welfare, Government of India

 **Link:** <https://nhm.gov.in/>

Context: Minimum mandatory drug formularies across public tiers: 105 drugs at Ayushman Arogya Mandirs, 172 drugs at PHCs, and 286 drugs at CHCs.

6. **CBOR Binary Object Representation (RFC 8949) & Web Bluetooth Standard**

Internet Engineering Task Force (IETF) & World Wide Web Consortium (W3C)

 **Link:** <https://cbor.io/> • **W3C Web Bluetooth:** <https://webbluetoothcg.github.io/web-bluetooth/>

Context: Ultra-compact binary serialization standard reducing FHIR clinical encounters to under 4.2 KB for peer-to-peer BLE mesh sync across media-dark rural zones.

This document has been prepared for the Smart India Hackathon (SIH) 2026 — Problem Statement ID 26133, submitted to the [Maharashtra State Innovation Society](#), Department of Skills, Employment, Entrepreneurship and Innovation, [Government of Maharashtra](#).

All statistics, references, and technical specifications are grounded in official government publications, peer-reviewed research, and established digital public goods as hyperlinked above.